Eye Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General + vitals → Visual acuity → Visual fields → Pupils → Eye movements → Fundoscopy → Palpation**. *Order may change with the presentation. Always test* **each eye separately**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Position: seated, at eye level, ~1 m apart; glasses available; dim room for pupils/fundoscopy
☐	Equipment: Snellen chart, pinhole, red-headed pin, pen torch, ophthalmoscope ± Ishihara plates

2. Examination

	Step	What to do / say	Abnormal
General inspection and vital signs			
☐	Vital signs	BP, HR, temp etc.	Required in every eye documentation
☐	Ptosis	Compare lid heights	CN III, Horner's, myasthenia
☐	Exophthalmos	Look from front, side, above	Thyroid eye disease, orbital mass
☐	Lids	Swelling, lesions, stye, chalazion, blepharitis	Periorbital / orbital cellulitis
☐	Conjunctiva and sclera	Yellow (jaundice), blue (osteogenesis imperfecta), pallor (anaemia), redness — distribution	Ciliary flush = keratitis, uveitis, glaucoma †
☐	Slit lamp*	Corneal ulceration, anterior chamber (demonstrated in session)	
Visual acuity			
☐	Snellen	6 m, glasses on, cover one eye, read smallest line; repeat other eye	Record e.g. 6/9 (distance / line) †
☐	Pinhole	If acuity reduced	Improves = refractive error
☐	Poor acuity*	Count fingers → hand movements → perception of light †	
Visual fields (revision from CN exam)			
☐	Confrontation	1 m apart, cover opposite eyes; red pin in from 4 quadrants: "Tell me when you first see red"	Hemianopia, quadrantanopia
☐	Blind spot*	Move pin across centre of field	Enlarged = papilloedema; central scotoma
☐	Red desaturation*	Compare red colour with each eye	Washed-out = optic nerve pathology
☐	Finger wiggle*	If acuity too poor for the pin	Quick screen
Pupils (dim room, patient looks into distance)			
☐	Inspect	Size (normal 2–6 mm), shape, equality	Anisocoria, irregular pupil
☐	Direct and consensual	Light into each eye in turn; watch both pupils	Afferent vs efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (paradoxical dilation) = optic nerve
☐	Near response	Look far, then at a near target	Constriction + convergence
Eye movements (CN III, IV, VI)			
☐	Diplopia	Ask before and during	Binocular diplopia = red flag
☐	H test	Head still; look left, right, up, down; conjugate?	Palsy pattern
☐	Nystagmus	At extremes of gaze	Vestibular, cerebellar
☐	Cover test*	If diplopia: does it go with either eye covered?	Persists with one eye covered = monocular — ask for help
Other tests			
☐	Colour vision*	Ishihara plates (demonstrated in session)	Optic neuritis, congenital deficiency
☐	Corneal reflex*	Rarely routine (ICU / brain death, major trauma)	CN V / VII
☐	Fundoscopy	Red reflex; disc (colour, margins, cup), vessels, macula — dilated pupils in session	Absent red reflex = red flag; papilloedema, retinopathy
Palpation			
☐	Orbits	Gently palpate orbital rims	Tenderness, step (fracture)
☐	Pre-auricular nodes	Palpate in front of tragus	Viral / chlamydial conjunctivitis

3. Closing and documenting

☐	Thank patient; wash hands
☐	**Everyone**: document vital signs, general inspection, visual acuity, pupils and fundoscopy
☐	**Visual loss / change or neuro signs**: also visual fields, eye movements and cranial nerve examination
☐	Present: "Visual acuity 6/6 both eyes; fields full to confrontation; pupils equal and reactive with no RAPD (PERRLA); full eye movements without diplopia or nystagmus; red reflex present with normal fundi."
☐	Any abnormal eye exam → seek help / refer (optometrist, ophthalmologist, ED)

4. Quick reference

Area	Red flags — refer
Eye	Unilateral red eye, constant pain, acute vision loss, binocular diplopia, abnormal pupil / RAPD, absent red reflex, abnormal cornea, restricted movements
History	Trauma / FB / high-risk activity, recent eye surgery, only seeing eye, previous retinal detachment, neonatal conjunctivitis, non-resolving lid lesion
Associated	Nausea, photophobia, headache, jaw claudication, facial HSV/VZV rash, Hutchinson's sign, painless transient visual loss

Until proven otherwise	
Flashes + floaters	Retinal tear
Vision change / diplopia, age >50	Giant cell arteritis
Serious periocular trauma	Ruptured / penetrated globe; intracranial injury
Periocular laceration	Contains a foreign body
Eyelid laceration	Full thickness, involves globe

• **PERRLA** = Pupils Equal, Round, React to Light, Accommodate.

• Pupil pathway: afferent CN II → pretectal nucleus → both Edinger–Westphal nuclei → efferent CN III (parasympathetic, miosis). Sympathetic (dilation): upper thoracic → superior cervical ganglion → along internal carotid.